

PACKAGE INSERT

SUN-GLIZIDE TABLET 80MG

Each tablet contains: Glizidate 80mg

Pharmacology
Glizidate is a hypoglycaemic sulphonylurea which stimulates secretion of insulin by functioning islet beta cells. The mechanism of action is mainly by the restoration of the early phase insulin secretion, resulting in a physiological release of insulin.

It has been shown that there is an improvement in the metabolic utilization of glucose at the peripheral level with the administration of glizidate. This extrapancreatic action may be due to the potentiation of the post-receptor pathways and is not due to a modification in the number of insulin receptors. Thus, glizidate restores glycaemic control throughout 24 hours. It normalizes the glycaemia in the fasting state and eliminates post-prandial hyperglycaemia peaks or reactional hypoglycaemia.

Glizidate is well absorbed and peak plasma concentrations occur 2-4 hours after administration. Glizidate is 85% bound to plasma proteins. Metabolism is extensive and all the metabolites are devoid of hypoglycaemic activity. 60-70% of the dose is excreted in the urine, <5% is excreted unchanged in the urine and 10-20% in the faeces as metabolites. The elimination half-life of glizidate is 10-12 hours.

Indications
Non-insulin dependent diabetes mellitus.

Side-effects / Adverse reactions
Skin sensitization, gastro-intestinal disturbances, leucopenia, intolerance to alcohol and jaundice may occur. In the event of sore throat or fever, repeated white cell counts should be carried out because abnormalities in the circulation blood may not appear for several days.

Hypoglycaemia occurs with all hypoglycaemic sulphonylureas. This potentially life-threatening complication requires prolonged and energetic treatment.

Symptoms of mild hypoglycaemia are sweating, pallor, intense hunger, sensation of malaise and tachycardia. In more severe case nervousness, hypothermia, headache, confusion and sometimes, convulsion and coma. In the conscious and co-operative patient, hypoglycaemia should be treated by the oral administration of a ready-absorbable form of carbohydrate such as sugar lumps or glucose based drink.

Precautions / Warnings
Care should be exercised in patients with hepatic and/or renal impairment and a small starting dose should be used with careful patient monitoring. It may be necessary even to withdraw any kind of oral antidiabetic treatment. It is possible to administer glizidate with anticoagulants, provided that haematological control and monitoring is frequent.
As with other sulphonylureas, hypoglycaemia will occur if the patient's dietary intake is reduced or if they are receiving a larger dose of glizidate than required.

Use in pregnancy and lactation
Insulin is preferred for the treatment of pregnant patients. Safe conditions for the use of glizidate in pregnancy has not been established and serious consideration should be given to the potential hazard of its use in women of childbearing age who may become pregnant.

Contraindications
It should not be used alone in insulin-dependent diabetes; diabetes complicated by ketosis and acidosis; diabetics undergoing surgery, after severe trauma or during infections; patients with known hypersensitivity to other sulphonylureas and related drugs.

Sun-Glize is contraindicated to used in coma and precoma diabetes, severe renal and hepatic dysfunction, pregnancy and lactation.

Dosage
Oral administration -
The dose should be adjusted according to the individual patient's response.
The usual initial dose is 40-80mg daily, gradually increased, if necessary up to 320mg daily.
A single dose should not exceed 2 tablets (160mg) and when higher doses are required, a twice-daily split is advised and according to the main meals of the day.

Drug Interactions
Compounds that may diminish the hypoglycaemic effect and thus necessitate an increase in the dosage requirement of the sulphonylurea include rifampicin, thiazide diuretics, adrenaline, chlorpromazine, corticosteroids, diazoxide, estrogens, oral contraceptives, phenothiazine derivatives and abuse of laxatives.

Compounds that potentate the hypoglycaemic effect of sulphonylureas and necessitate a reduction in their dosage requirement include anti-infective agents such as chloramphenicol, fluconazole, miconazole, sulphonamides; some anti-inflammatory agents and analgesics including azapropazone; phenylbutazone, salicylates; thyroid hormones; coumarin anticoagulants; heparin; halofenate; monoamine, oxidase inhibitors; tetracycline compounds; disopyramide and ulcer healing drugs such as cimetidine, which are also found to enhance the hypoglycaemic effect of sulphonylureas.

Clofibrate group, for example gemfibrozil, which may improve glucose tolerances, found to have an additive effect.

Beta-blockers may mask some of the symptoms of hypoglycaemia (e.g. tremor). There has been a report of beta-blockers reducing the hypoglycaemic action of glizidate, also it should be remembered that beta-blockers may have hypoglycaemic or hyperglycaemic actions on their own.

Symptoms and treatment for overdose and antidotes
The symptom to be expected of overdose would be hypoglycaemia. The treatment is gastric lavage and correction of the hypoglycaemia by appropriate means with continued monitoring of the patient's blood sugar until the effect of the drug has ceased.

Storage conditions
Store below 30°C. Protect from light.

Shelf-life: 3 years

Pack size: Blister pack: A box of 100 x 10 tablets per strip.
50 x 10 tablets per strip.
10 x 10 tablets per strip.

Pack size (export only): A bottle of 500 tablets.

Description: Round, flat, white, plain tablet with single score on one side only.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN/LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:
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